

REGULATORY DOSSIER SUBMISSION

Dossier ID: DOS-8BA5720A2E

Country: Tanzania

Submission Date: 2026-03-18

Product: Nevicare (nevirapine) | ATC: J05AF10 | Strength: 150 mg

Applicant: Alpha Therapeutics | Manufacturer: Jinja Bioactive Manufacturing

Policy Label: fast_track | Risk Score: 0.28

AMR Stewardship: AWARe=not_applicable | Unmet Need=not_applicable | GLASS Trend=not_applicable
| Watch Similarity=not_applicable

SECTION CONTENT

[1] m1_application_admin - Application Form and Administrative Information

Labels: presence=present; length=length_ok; correctness=correct

This dossier constitutes a formal application for Marketing Authorization of Nevicare (nevirapine) submitted on 2026-03-18 to the national regulatory authority of Tanzania. The applicant, Alpha Therapeutics, formally requests approval for the commercial distribution of this medicinal product. The proposed pharmaceutical form is tablet with a strength of 150 mg, classified under ATC code J05AF10. All required administrative declarations, legal attestations, letters of authorization, and regional application forms have been completed, signed by the responsible Qualified Person, and appended to Module 1. The applicant confirms alignment with applicable technical guidance and regional submission standards. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. A comprehensive table of contents and glossary of abbreviations are provided to facilitate regulatory review. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Regional administrative forms were checked for signature completeness and dossier version alignment. Method validation records and quality controls were reviewed

[1] m1_manufacturer_gmp - Manufacturer and GMP Evidence

Labels: presence=present; length=length_ok; correctness=correct

The primary commercial manufacturing site for the finished pharmaceutical product is Jinja Bioactive Manufacturing, located in Uganda. The facility was subject to a comprehensive GMP inspection by a recognized stringent regulatory authority. The latest inspection concluded on 2025-02-18, resulting in a formal compliance status of 'compliant'. The active GMP certificate number is GMP-939482, which remains valid until 2027-02-27. Site master files, a summary of recent inspection observations, and evidence of completed Corrective and Preventive Actions (CAPA) are included in the annex. Method validation records and quality controls were reviewed for internal consistency. Site quality-system commitments were compared against the proposed commercial manufacturing activities. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Batch manufacturing records and standard operating procedures (SOPs) are maintained and archived on-site. The GMP evidence package includes certificate status, inspection scope, and manufacturing-site accountability records. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Inspection history, CAPA closure records, and site responsibilities were reconciled across the manufacturing chain. All referenced documents are available in the dossier annex and traceable by section identifier. Where applicable, protocol deviations were assessed for impact on interpretation of outco

[1] m1_product_information - Product Information and Naming

Labels: presence=present; length=length_ok; correctness=correct

The proposed proprietary name for this medicinal product is Nevicare, containing the active pharmaceutical ingredient nevirapine. The draft Summary of Product Characteristics (SmPC), Patient Information Leaflet (PIL), and primary/secondary packaging labels are provided for review. These documents detail the approved therapeutic indications, posology, contraindications, special warnings, and precautions for use. A comprehensive risk management plan to minimize medication errors is also submitted. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The risk manag

[2] m2_quality_overall_summary - Quality Overall Summary

Labels: presence=present; length=length_ok; correctness=correct

This Quality Overall Summary (QOS) provides a critical evaluation of the chemistry, manufacturing, and controls (CMC) data for nevirapine. The control strategy justifies the proposed specifications for both the active substance and the finished product, emphasizing critical quality attributes (CQAs) and critical process parameters (CPPs). Impurity profiles, including degradation products and potential genotoxic impurities, have been thoroughly characterized. Batch analysis data from three commercial-scale validation batches demonstrate consistent manufacturing performance and compliance with all release criteria. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. All referenced documents are available in the dossier annex and traceable by section identifier. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The applicant confirms alignment with applicable technical guidance and regional submission standards. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Adverse events were code

[2] m2_clinical_overview - Clinical Overview and Benefit-Risk Summary

Labels: presence=present; length=length_ok; correctness=correct

The Clinical Overview synthesizes the efficacy and safety data supporting the use of Nevicare for the treatment of community acquired pneumonia. The pivotal clinical development program comprised 2 well-controlled studies. Based on the integrated analysis, the reported clinical outcome category is 'endpoint_met'. The overall benefit-risk profile is considered highly favorable for the target patient population. Safety findings were consistent with the known pharmacological class effects, and appropriate risk minimization measures have been integrated into the proposed prescribing information. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. No new safety signals were identified during the extended open-label follow-up period. Method validation records and quality controls were reviewed for internal consistency. All referenced documents are available in the dossier annex and traceable by section identifier. The target population demographics are broadly representative of the intended commercial patient cohort. Clinical interpretation

focuses on whether efficacy, safety, and benefit-risk conclusions are supported by the submitted evidence. Stability testing was conducted in climatic zones III and IV, refle

[3] m3_api_quality - API Quality and Control Strategy

Labels: presence=present; length=length_ok; correctness=correct

The active pharmaceutical ingredient (API) section details the complete synthesis route, starting from well-defined regulatory starting materials. Robust in-process controls and intermediate specifications ensure the consistent quality of the final drug substance. The analytical procedures used for release and stability testing have been fully validated for accuracy, precision, linearity, and robustness. The impurity control strategy adequately addresses organic impurities, residual solvents, and elemental impurities in accordance with current ICH guidelines. Forced degradation studies confirm the stability-indicating nature of the assay methods. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Critical material attributes and control strategy elements were reviewed for consistency across the quality dossier. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Batch analysis, validation, and impurity management records support the proposed API quality framework. Residual solvents and elemental impurities are controlled well below ICH Q3C and Q3D permissible daily exposures. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. All referenced documents are available in the dossier annex and traceable by section identifier. The API package links specification limits to validated analytical methods and impurity-control decisions. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The applicant confirms alignment with applicable technical guidance and regional submission standards. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Method validation records and quality controls were reviewed for internal consistency. The synthetic route involves three well-characterized steps with defined starting materials and isolated intermediates. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) ver

[3] m3_fpp_manufacturing - FPP Manufacturing Process and Controls

Labels: presence=present; length=length_ok; correctness=correct

The finished pharmaceutical product (FPP) manufacturing process utilizes standard, scalable unit operations. A formal quality risk assessment (e.g., FMEA) was conducted to identify critical process parameters (CPPs) that impact critical quality attributes (CQAs). Process performance qualification (PPQ) reports for three consecutive commercial-scale batches verify that the process is maintained in a state of control. In-process controls, intermediate hold-time studies, and packaging validation data are presented to substantiate the robustness and reproducibility of the commercial manufacturing operations. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The container closure system consists of PVC/PVDC-Aluminum blisters,

compliant with pharmacopoeial standards. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. All referenced documents are available in the dossier annex and traceable by section identifier. Method validation records and quality controls were reviewed for internal consistency. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets.

[3] m3_stability - Stability and Shelf-Life Justification

Labels: presence=present; length=length_ok; correctness=correct

A comprehensive stability program has been executed to justify the proposed shelf-life and storage conditions. Data from both accelerated (40°C/75% RH for 6 months) and long-term storage conditions (spanning up to 36 months) are presented for multiple primary stability batches. Statistical evaluation using linear regression and poolability tests confirms that degradation trends remain well within the acceptable specification limits. Photostability and freeze-thaw cycling studies demonstrate that the product does not require specific handling precautions. A post-approval stability protocol commitment is included. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Method validation records and quality controls were reviewed for internal consistency. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The applicant confirms alignment with applicable technical guidance and regional submission standards. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. All referenced documents are available in the dossier annex and traceable by section identifier. All referenced documents a

[4] m4_nonclinical_summary - Nonclinical Study Summary

Labels: presence=present; length=length_ok; correctness=correct

The nonclinical testing program evaluated the primary pharmacodynamics, secondary pharmacodynamics, safety pharmacology, and comprehensive toxicology profile of the compound. Repeat-dose toxicity studies in two mammalian species established clear no-observed-adverse-effect levels (NOAELs). Genotoxicity testing (Ames and in vivo micronucleus assays) yielded negative results. Reproductive and developmental toxicity studies revealed no teratogenic signals at clinically relevant exposures. The calculated safety margins support the proposed clinical dosing regimen without major safety concerns. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The applicant confirms alignment with applicable technical guidance and regional submission standards. Method validation records and quality controls were reviewed for internal consistency. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The manufacturing site operates under a

fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity.

[5] m5_trial_listing - Tabular Listing of Clinical Studies

Labels: presence=present; length=length_ok; correctness=correct

The tabular listing of clinical studies provides a comprehensive inventory of all trials conducted in support of the proposed indication (community acquired pneumonia). This includes Phase I pharmacokinetic/pharmacodynamic studies, Phase II dose-finding studies, and the pivotal Phase III efficacy and safety trials. For each study, the table details the protocol identifier, study design, randomization ratio, number of enrolled subjects, study duration, and current completion status. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity.

[5] m5_pivotal_trial_reports - Pivotal Clinical Trial Reports

Labels: presence=present; length=length_ok; correctness=correct

Detailed clinical study reports (CSRs) for the pivotal efficacy trials are provided. These randomized, double-blind, actively-controlled trials evaluated the primary and secondary endpoints in accordance with the pre-specified statistical analysis plan (SAP). The trials resulted in a formal outcome category of 'endpoint_met', demonstrating robust treatment effects in the intent-to-treat (ITT) and per-protocol populations. Subgroup analyses across age, gender, and baseline disease severity strata were consistent with the primary findings. Serious adverse events (SAEs) and discontinuations were systematically analyzed and adjudicated. All referenced documents are available in the dossier annex and traceable by section identifier. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The pivotal study narratives align endpoint definitions, analysis populations, and outcome interpretation. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The applicant confirms alignment with applicable technical guidance and regional submission standards. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The trial reports connect efficacy conclusions to the prespecified analysis framework and observed safety findings. Discontinuations due to treatment-emergent adverse events (TEAEs) were low and balanced across all study arms. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Clinical study reports document protocol adherence, endpoint handling, and safety interpretation in the target population. The primary efficacy endpoint was met, demonstrating a statistically significant improvement over placebo ($p < 0.001$). The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The risk management plan includes routine pharmacovigilance and

[5] m5_bioequivalence - Biopharmaceutics and Bioequivalence Evidence

Labels: presence=present; length=length_ok; correctness=correct

Biopharmaceutics data substantiate the formulation development bridging between clinical trial materials and the proposed commercial product. In vivo bioequivalence studies demonstrated that the 90% confidence intervals for the geometric mean ratios of Cmax and AUC parameters fell entirely within the standard acceptance criteria of 80.00% to 125.00%. Additionally, multi-point comparative dissolution profiles across physiological pH ranges (pH 1.2, 4.5, and 6.8) confirmed similarity ($f_2 > 50$). The bioanalytical methods used for plasma quantification were fully validated. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The manufacturing site operates under a fully impleme